

Home Health Care Utilization Among U.S. Cancer Survivors: Evidence from the Medical Expenditure Panel Survey (2016–2019)

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Abstract

Cancer survivorship in the United States is expanding, and many survivors manage treatment sequelae and functional limitations at home. We used nationally representative data from the Medical Expenditure Panel Survey (MEPS, 2016–2019) to characterize home health care (HHC) among adult cancer survivors, describing prevalence, intensity (days), and payer mix, and identifying predictors of each. Analyses were survey-weighted using pooled, annualized person weights and year-specific strata/PSU interactions; pooled models included calendar-year fixed effects. We fit logistic models for any HHC use, quasi-Poisson models for days among users, and log-linear models for payments among users with positive expenditures. Approximately one in ten survivors used HHC in a given year (9.76%, SE 0.43%). Among users, episodes averaged 82.0 days (SE 6.7) and were dominated by agency-provided care. Financing was predominantly public: mean Medicare-paid amounts among survivor-users were approximately \$5,129 and Medicaid approximately \$2,967, with private insurance and out-of-pocket payments contributing much smaller shares. In adjusted models, older age and socioeconomic vulnerability (public coverage, lower income, lower education, and non-married status) were consistently associated with greater likelihood of use, higher intensity, and higher payments; uninsured survivors had markedly fewer days and substantially lower spending. A combined interaction model suggested that insurance gradients in any use were

attenuated among cancer survivors relative to non-cancer adults, but strong gradients re-emerged for intensity and payments. These findings indicate that structural and insurance factors shape HHC more powerfully than cancer status alone and argue for policies that stabilize public coverage and proactively target high-need survivors.

1 Introduction

Survival after cancer diagnosis has improved steadily, shifting the clinical focus toward long-term management of chronic symptoms, late effects, and disability in everyday settings. Home health care can deliver nursing, rehabilitation, and supportive services where survivors live, but access and intensity are not uniform. Coverage rules, benefit design, and patients' socioeconomic context often determine who receives HHC and how much of it. Yet national evidence that simultaneously describes who uses HHC, how intensively they use it, and who pays for it across the full adult survivorship continuum remains limited. Leveraging four years of the MEPS Household Component, we provide an integrated, survey-weighted portrait of HHC among U.S. adult cancer survivors, quantify payer contributions, and estimate adjusted associations between socio-demographic factors and HHC use, intensity, and payments.

2 Methods

2.1 Data source and population

We analyzed the 2016–2019 MEPS Full-Year Consolidated (FYC) files, which provide person-level HHC variables and payer-specific totals, thereby avoiding double counting from event-level joins. Adults aged ≥ 18 years were included. Cancer survivors were identified using site-specific history variables when available, excluding non-melanomatous skin cancer when such flags were present; this approach captured survivors across the continuum from active treatment to long-term survivorship.

2.2 Outcomes

We examined three outcomes. First, *any HHC use* (yes/no) based on total HHC days > 0 . Second, *intensity* as total HHC days among users. Third, *payments* (agency plus non-agency) among users with positive expenditures, overall and by payer (Medicare, Medicaid, private, and out-of-pocket). We additionally summarized *charges* and calculated the charges-to-payments ratio.

2.3 Covariates

Covariates included age (continuous), sex, race/ethnicity (White, Black, Native, Asian/Pacific Islander, Multiple), insurance status (Private, Public-only, Uninsured), poverty category (Poor, Near-poor, Low-income, Middle, High), education (No high school [HS], GED, HS, some college, AA, BA, MA/above), and marital status (Married, Widowed, Divorced, Separated, Never married).

2.4 Survey design and pooling

We annualized the FYC person weight by dividing by four and created year-specific strata and primary sampling unit interactions to avoid collisions when pooling years. All estimates used Taylor-series linearization with adjustments for lonely PSUs. Year-specific designs were used for time-trend figures; pooled designs were used for overall estimates and models. Pooled models included fixed effects for calendar year (2016–2019).

2.5 Statistical analysis

We first described prevalence and intensity overall and by covariate subgroups among cancer survivors, and summarized payer means among HHC users. We then fit three sets of survey-weighted models: (1) logistic regression for any HHC use; (2) quasi-Poisson regression with log link for days among users; and (3) linear models for $\log(\text{payments} + 1)$ among users with

spending > 0 . Models were stratified by cancer status (survivor vs. non-cancer adults), and we fit a combined interaction model including cancer status $\times$ covariates to formally probe differential associations by survivorship.

3 Results

3.1 Prevalence of any HHC

In pooled analyses, 9.76% (SE 0.43%) of cancer survivors used HHC in a given year. Women showed slightly higher prevalence than men (10.37% vs 8.84%). Prevalence differed by race/ethnicity, ranging from 12.33% among Black survivors to 4.75% among Asian/Pacific Islander survivors. Insurance status was strongly patterned: 14.36% among publicly insured survivors, 7.33% among those with private insurance, and 2.19% among the uninsured. Poverty and education gradients were pronounced: 17.85% among those living in poverty versus 5.08% in the highest income category; 20.78% among survivors without a high-school diploma versus 5.51% among those with a bachelor's degree. Widowed survivors reported the highest prevalence (19.88%) vs married survivors (6.61%). These patterns are summarized in Figure 1, the year trends in Figure 2, and socioeconomic gradients in Figure 3 and Figure 4.

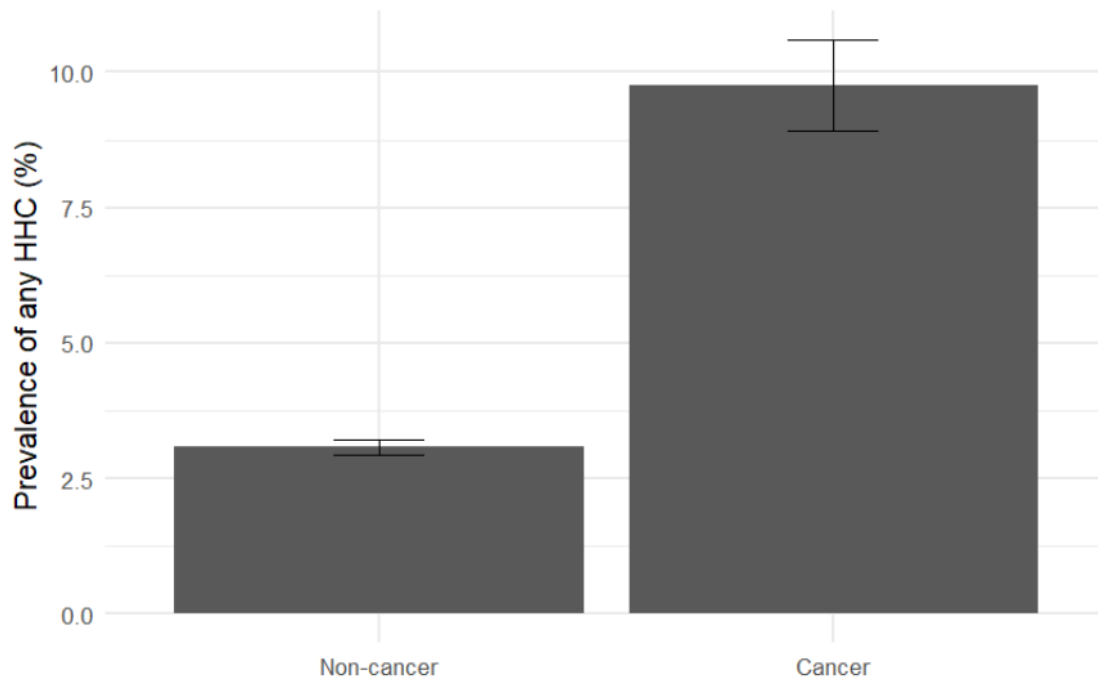


Figure 1: Any home health care, pooled 2016–2019 (cancer vs. non-cancer). Survey-weighted prevalence with 95% CIs.

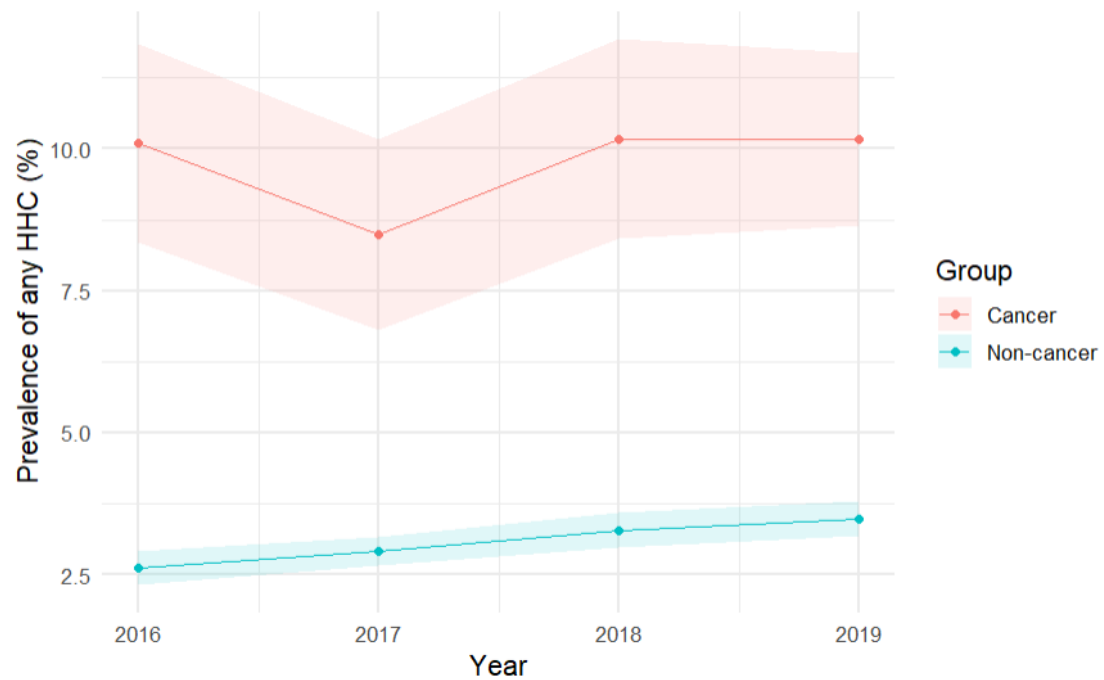


Figure 2: Any home health care by year and cancer status, 2016–2019, with 95% CIs.

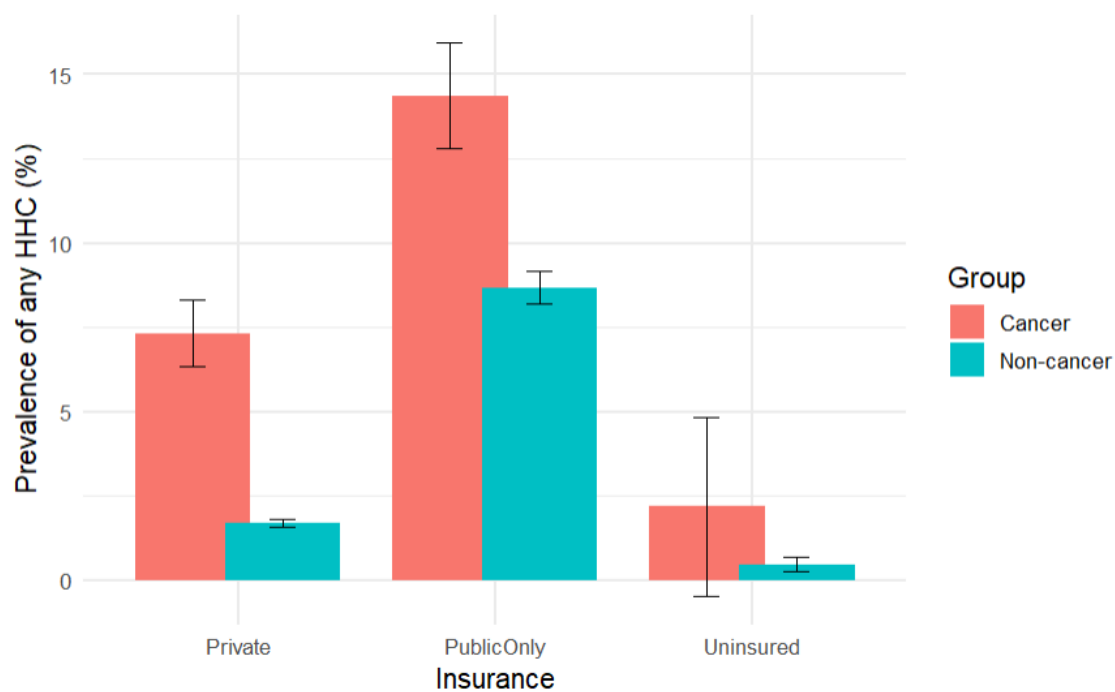


Figure 3: Prevalence of any HHC by insurance category, stratified by cancer status.

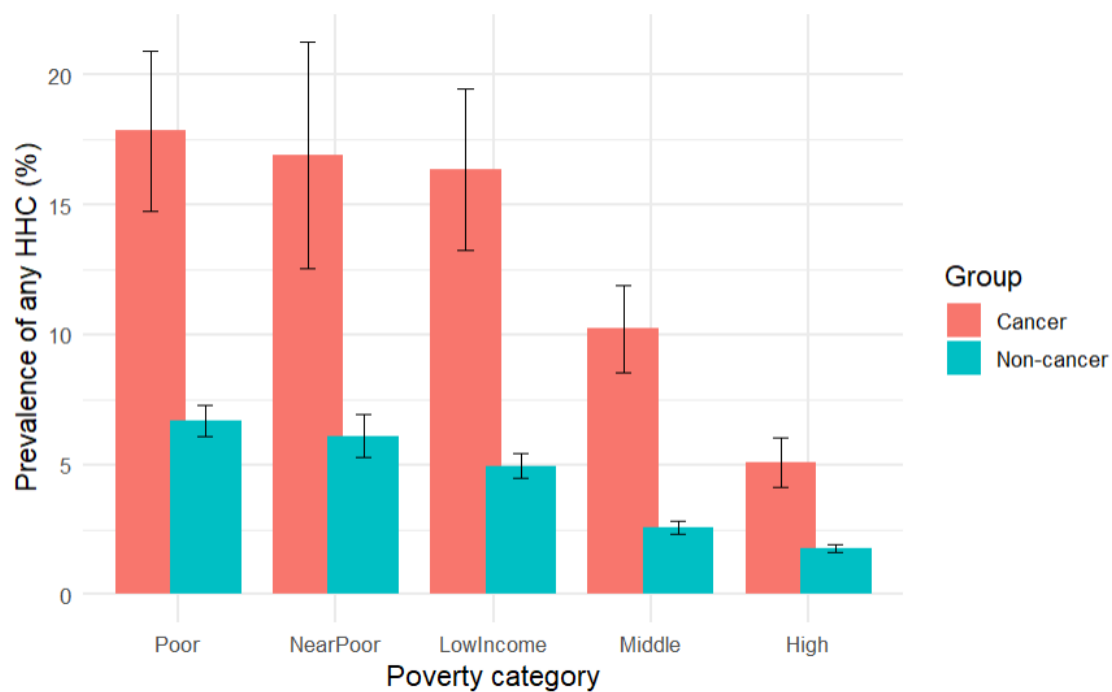


Figure 4: Prevalence of any HHC by poverty category, stratified by cancer status.

3.2 Intensity (days) among users

Among cancer survivors who used HHC, the mean number of days was 82.0 (SE 6.7). Agency-provided days dominated (68.8), with smaller contributions from self-employed providers (3.3) and unpaid informal caregivers not living with the participant (9.9). Intensity differed by subgroup: Black 105.9 vs White 78.6 vs Asian/PI (Asian/Pacific Islander) 27.4 days; Public-only 108.4, Private 52.9, Uninsured 18.2; by poverty, Poor 117.7 vs High 55.7; by education, No HS (No high school degree) 134.5 vs BA 16.1; and by marital status, Widowed 88.2 vs Married 73.9 (Tables B1–B7). These descriptive patterns are directionally consistent with the adjusted intensity estimates in Table A3–A4 and the forest plot in Figure C2.

3.3 Payments and payer mix among users

Among survivor-users, mean agency payments were \$9,196 and non-agency \$167, yielding \$9,363 total per user. Medicare contributed the largest mean amount (\$5,129), followed by Medicaid (\$2,967); out-of-pocket and private payments were much smaller (\$201 and \$257, respectively, combining agency and non-agency). Year-to-year patterns and composition are shown in Figure 5 and the stacked payer mix in Figure 6; agency vs non-agency contributions (Medicare focus) in Figure 7. Charges exceeded payments throughout, so the charges-to-payments ratio > 1 (Figure 8).

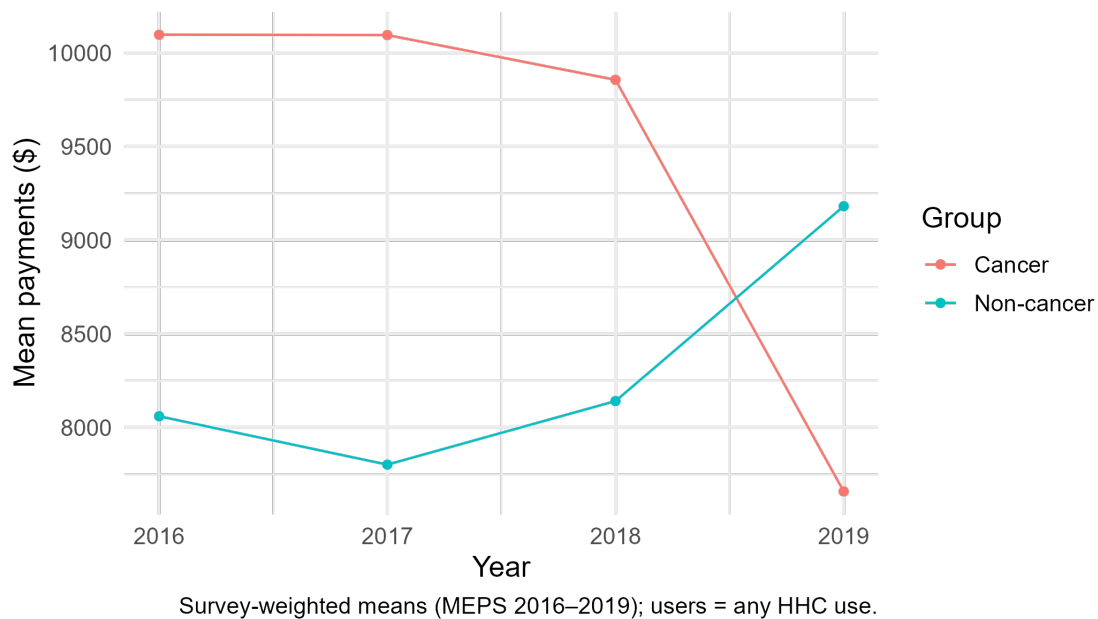


Figure 5: Total HHC payments among users by year and cancer status (2016–2019).

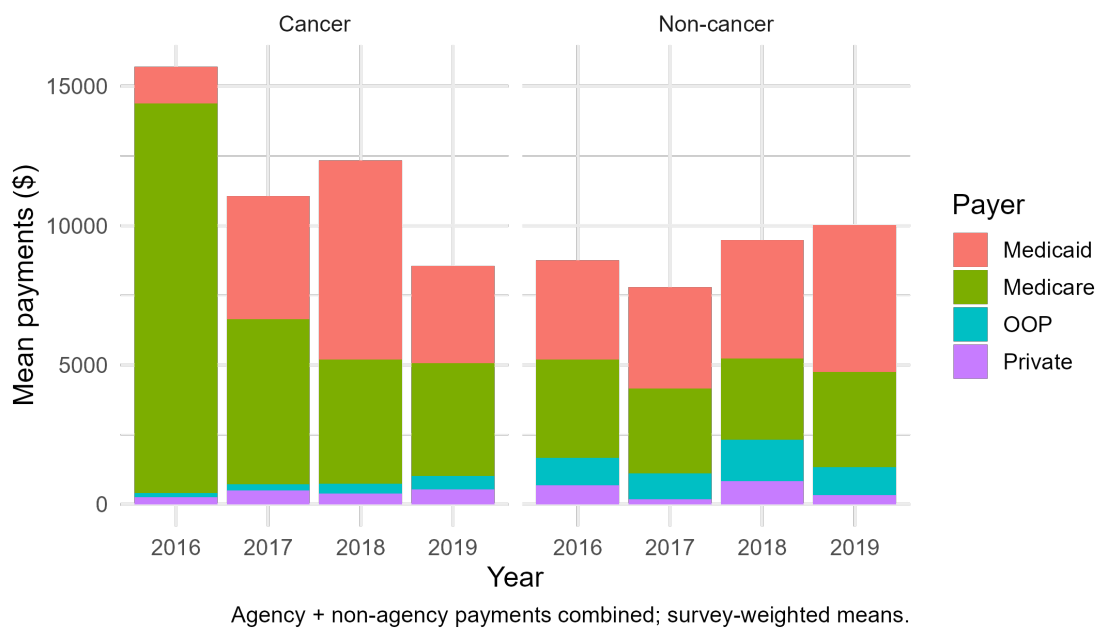
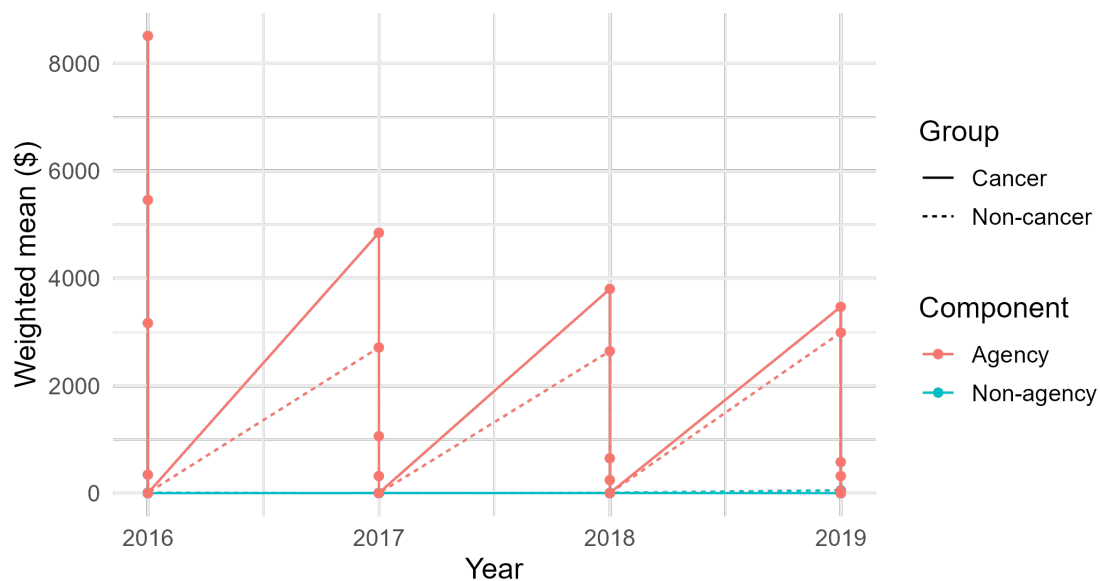
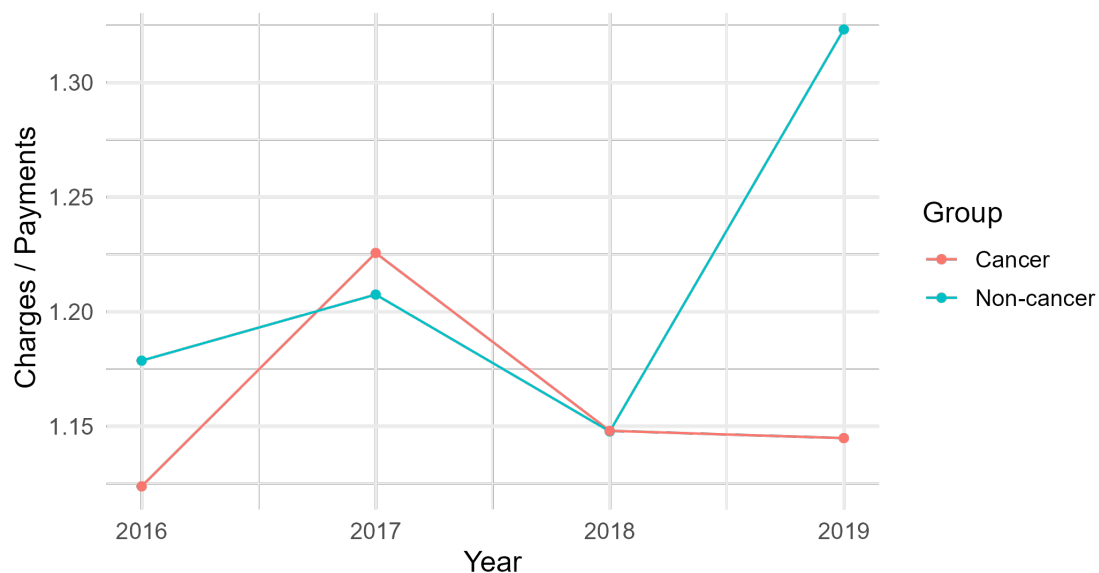


Figure 6: Payer mix among HHC users by year and cancer status (stacked bars for Medicare, Medicaid, private, and out-of-pocket).



Survey-weighted means by cancer status among HHC users.

Figure 7: Agency (HHAEXP) vs. non-agency (HHNEXP) payments over time among users, stratified by cancer status; Medicare focus.



Total charges = HHATCH + HHNTCH; payments = HHAEXP + HHNEXP.

Figure 8: Charges-to-payments ratio among HHC users by year and cancer status.

In pooled comparisons, survivor-users had higher mean total payments than non-cancer users (\$10,839 vs \$10,192), and a greater Medicare share per user; Medicaid’s contribution remained substantial in both groups.

3.4 Models

Usage of HHC (logistic). Among survivors, each year of age increased the odds of any HHC use by about 5.9% (OR 1.059; $p < 0.001$). Race/ethnicity differences were mixed after adjustment; Asian/Pacific Islander survivors had lower odds (OR 0.486; $p = 0.055$, borderline), while those reporting multiple races had higher odds (OR 1.987; $p = 0.019$). Compared with those living in poverty, higher-income survivors had lower odds (Middle OR 0.554; High OR 0.358; both $p \leq 0.001$). Lower education was associated with higher odds of any use (e.g., HS vs. No HS OR 0.584; $p < 0.001$). Widowed (OR 1.467; $p = 0.009$) and never-married survivors (OR 2.344; $p < 0.001$) had higher odds than married survivors. In contrast to non-cancer adults, public-only coverage among survivors did not independently raise the odds of any use (OR 1.102; $p = 0.364$), while uninsured survivors had lower odds with wide uncertainty (OR 0.365; $p = 0.144$). Full coefficients are in Table A1 and Table A2; the side-by-side forest plot is in Figure C1. A combined interaction model confirmed attenuation of the public-only insurance effect on any use among survivors (cancer $\times$ Public-only $\beta = -0.621$; $p < 0.001$).

Intensity (quasi-Poisson). Among survivor-users, days increased with age (rate ratio [RR] 1.016 per year; $p = 0.005$). Public-only coverage was associated with 68.7% more days (RR 1.687; $p = 0.008$), while being uninsured was associated with 65.5% fewer days (RR 0.345; $p < 0.001$). Intensity decreased with income (Middle RR 0.606; High RR 0.638; both $p \leq 0.032$) and with education (e.g., HS vs. No HS RR 0.613; $p = 0.005$; BA vs. No HS RR 0.207; $p < 0.001$). Asian/Pacific Islander survivors had substantially fewer days than White survivors (RR 0.211; $p = 0.001$). Corresponding results for non-cancer users show higher intensity for Black adults and public-only coverage, and fewer days for the uninsured

(Table A4).

Payments (log-linear among users with > 0). Among survivor-users, each year of age was associated with a 2.5% increase in payments. Public-only coverage predicted a large increase relative to private coverage, whereas being uninsured predicted a substantial decrease. Payments declined with income (e.g., High vs. Poor approximately -49%) and with educational attainment relative to No HS (e.g., HS approximately -31%). Among non-cancer users, public-only coverage predicted a sizable increase, being uninsured a decrease, female sex a decrease, and non-married categories an increase relative to married adults. Full results are shown in Table A5 and Table A6; see also the forest plot in Figure C3.

To aid interpretation, we display the adjusted effects as forest plots in Figure C1–Figure C3.

4 Discussion

This national analysis shows that HHC among adult cancer survivors is both selective and intensive. Only about one in ten survivors used HHC in a given year, yet, conditional on use, episodes were prolonged, with a mean of 82 days and a distribution dominated by agency-provided care. Utilization and intensity were concentrated among socioeconomically vulnerable survivors—those with public coverage, lower income, less education, and without a spouse—patterns that persisted after adjustment (see Table A1, Table A3, and Table A5). Financing was predominantly public, with Medicare paying the largest share per user and Medicaid providing a substantial and variable contribution; private and out-of-pocket payments were comparatively small (Figure 6). Charges exceeded payments in all years (Figure 8).

The models clarify how structural factors operate at distinct decision points. Among survivors, the presence of cancer may compress some coverage differences in the extensive margin (any use) relative to non-cancer adults, as suggested by the attenuated public-only effect in the interaction model and the utilization comparisons in Table A1–Table A2. However,

once a survivor is an HHC user, public coverage is strongly associated with more days and higher payments (Table A3, Table A5), while being uninsured is associated with substantially fewer days and markedly lower spending. Income and education function as cross-cutting determinants across the extensive and intensive margins, consistent with the patterns in Figure 4, Table A3, and Table A5.

Racial and ethnic patterns were heterogeneous: Asian/Pacific Islander survivors had fewer days than White survivors (quasi-Poisson model; Table A3), while Black adults in the non-cancer group had more days than White adults (Table A4). Time trends highlight a decline in survivor payments from 2016 to 2019 (Figure 5), with a simultaneous increase in the charges-to-payments ratio (Figure 8).

5 Policy and Practice Implications

Because public coverage underwrites most HHC for survivors, policy stability in Medicare and Medicaid is foundational to survivorship care. Gaps in coverage are consequential: uninsured survivors used far fewer HHC days and generated substantially lower spending (see Table A3 and Table A5), suggesting unmet need or substitution to informal care. Targeting high-need survivors—those in poverty, with limited education, and without spousal support—could improve equity and efficiency. Scalable approaches include proactive identification of high-risk survivors at oncology discharge, streamlined authorization for HHC, and enhanced care coordination to reduce fragmentation between acute care, oncology, and home health agencies.

6 Limitations

The analysis is cross-sectional within pooled years and cannot make causal claims. MEPS relies on household reports augmented by provider follow-back; misclassification is possible, though our use of FYC person-level totals mitigates double counting. Clinical detail on cancer stage, time since diagnosis, and treatment status is limited, so residual confounding

by clinical severity may remain. Although we excluded non-melanomatous skin cancer where feasible, site flags vary by year, and some misclassification is possible. Finally, charges and payments in MEPS reflect reported amounts and may not perfectly align with agency cost structures.

7 Conclusions

Home health care for U.S. cancer survivors is a selective but intensive service financed primarily by public payers. Structural factors—age, coverage, income, education, and marital status—shape who receives HHC, how much is delivered, and how much is paid, often more strongly than cancer status itself. Policies that stabilize and strengthen Medicare and Medicaid HHC benefits, prevent coverage gaps, and proactively target high-need survivors are likely to reduce inequities and improve survivorship outcomes.

Funding

No external funding was obtained for this work.

Competing Interests

The authors declare no competing interests.

Ethics

This study used publicly available, de-identified survey data and did not involve human subjects as defined by federal regulations.

Data and Code Availability

MEPS public-use files are available from AHRQ. The R code used to construct analytic files, designs, figures, and models is documented in the Methods and can be shared on request.

Appendix A: Model Tables

Table A1: Predictors of any home health care use among cancer adults (survey-weighted logistic regression).

Term	Adjusted OR (95% CI)	p-value
Age (years)	1.06 (1.05, 1.07)	0.000
Female vs Male	1.16 (0.95, 1.41)	0.150
Black vs White	1.09 (0.81, 1.48)	0.555
Native vs White	0.98 (0.32, 3.06)	0.978
Asian/PI vs White	0.49 (0.23, 1.01)	0.055
Multiple vs White	1.99 (1.12, 3.52)	0.019
Public-only vs Private	1.10 (0.89, 1.36)	0.364
Uninsured vs Private	0.37 (0.09, 1.41)	0.144
Near-poor vs Poor	0.74 (0.49, 1.13)	0.168
Low-income vs Poor	0.77 (0.55, 1.06)	0.112
Middle vs Poor	0.55 (0.41, 0.76)	0.000
High vs Poor	0.36 (0.25, 0.50)	0.000
GED vs No HS	0.76 (0.48, 1.22)	0.258
HS vs No HS	0.58 (0.45, 0.75)	0.000
Some college vs No HS	0.65 (0.45, 0.94)	0.021
AA vs No HS	0.42 (0.26, 0.69)	0.001
BA vs No HS	0.43 (0.22, 0.86)	0.016
MA vs No HS	0.55 (0.37, 0.83)	0.005
Widowed vs Married	1.47 (1.10, 1.95)	0.009
Divorced vs Married	1.32 (0.98, 1.79)	0.070
Separated vs Married	1.09 (0.54, 2.19)	0.803

Never married vs Married	2.34 (1.63, 3.37)	0.000
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Notes: Reference categories: Male, White, Private, Poor, No HS, Married. 95% CIs in parentheses.

Table A2: Predictors of any home health care use among non-cancer adults (survey-weighted logistic regression).

Term	Adjusted OR (95% CI)	p-value
Age (years)	1.06 (1.06, 1.07)	0.000
Female vs Male	1.25 (1.13, 1.38)	0.000
Black vs White	1.08 (0.95, 1.22)	0.243
Native vs White	1.07 (0.69, 1.66)	0.776
Asian/PI vs White	0.78 (0.59, 1.02)	0.066
Multiple vs White	1.32 (1.01, 1.73)	0.041
Public-only vs Private	2.05 (1.84, 2.28)	0.000
Uninsured vs Private	0.31 (0.20, 0.48)	0.000
Near-poor vs Poor	0.77 (0.64, 0.93)	0.006
Low-income vs Poor	0.68 (0.59, 0.78)	0.000
Middle vs Poor	0.53 (0.46, 0.62)	0.000
High vs Poor	0.46 (0.39, 0.55)	0.000
GED vs No HS	1.18 (0.96, 1.46)	0.118
HS vs No HS	0.85 (0.75, 0.97)	0.014
Some college vs No HS	0.72 (0.59, 0.87)	0.001
AA vs No HS	0.76 (0.60, 0.97)	0.025
BA vs No HS	0.82 (0.57, 1.17)	0.271
MA vs No HS	0.85 (0.70, 1.04)	0.115
Widowed vs Married	2.05 (1.77, 2.38)	0.000

Divorced vs Married	1.53 (1.33, 1.76)	0.000
Separated vs Married	2.30 (1.80, 2.94)	0.000
Never married vs Married	2.61 (2.22, 3.06)	0.000

Notes: Reference categories as above.

Table A3: Intensity (quasi-Poisson): adjusted rate ratios for total HHC days among users, cancer survivors.

Term	Adjusted RR (95% CI)	p-value
Age (years)	1.02 (1.00, 1.03)	0.005
Female vs Male	1.03 (0.78, 1.36)	0.821
Black vs White	1.12 (0.83, 1.50)	0.453
Native vs White	1.30 (0.55, 3.07)	0.551
Asian/PI vs White	0.21 (0.08, 0.54)	0.001
Multiple vs White	1.64 (0.94, 2.85)	0.079
Public-only vs Private	1.69 (1.15, 2.48)	0.008
Uninsured vs Private	0.34 (0.20, 0.58)	0.000
Near-poor vs Poor	1.20 (0.78, 1.85)	0.407
Low-income vs Poor	0.78 (0.51, 1.21)	0.273
Middle vs Poor	0.61 (0.40, 0.92)	0.019
High vs Poor	0.64 (0.42, 0.96)	0.032
GED vs No HS	0.59 (0.36, 0.96)	0.033
HS vs No HS	0.61 (0.44, 0.86)	0.005
Some college vs No HS	0.73 (0.46, 1.15)	0.175
AA vs No HS	0.63 (0.31, 1.30)	0.208
BA vs No HS	0.21 (0.09, 0.46)	0.000

MA vs No HS	0.74 (0.33, 1.70)	0.479
Widowed vs Married	0.82 (0.53, 1.29)	0.393
Divorced vs Married	0.65 (0.41, 1.04)	0.072
Separated vs Married	0.51 (0.17, 1.54)	0.227
Never married vs Married	1.23 (0.72, 2.10)	0.444

Notes: Reference categories as in Table A1.

Table A4: Intensity (quasi-Poisson): adjusted rate ratios for total HHC days among users, non-cancer adults.

Term	Adjusted RR (95% CI)	p-value
Age (years)	1.00 (1.00, 1.01)	0.270
Female vs Male	0.90 (0.80, 1.01)	0.078
Black vs White	1.23 (1.09, 1.39)	0.001
Native vs White	1.34 (0.85, 2.11)	0.210
Asian/PI vs White	1.04 (0.77, 1.40)	0.794
Multiple vs White	0.99 (0.76, 1.29)	0.944
Public-only vs Private	1.44 (1.22, 1.70)	0.000
Uninsured vs Private	0.49 (0.27, 0.90)	0.021
Near-poor vs Poor	0.88 (0.74, 1.06)	0.172
Low-income vs Poor	0.93 (0.80, 1.08)	0.312
Middle vs Poor	0.81 (0.69, 0.96)	0.018
High vs Poor	0.76 (0.63, 0.93)	0.007
GED vs No HS	0.57 (0.45, 0.72)	0.000
HS vs No HS	0.73 (0.64, 0.84)	0.000
Some college vs No HS	0.79 (0.62, 1.02)	0.066

AA vs No HS	0.56 (0.39, 0.78)	0.001
BA vs No HS	0.54 (0.32, 0.90)	0.020
MA vs No HS	0.69 (0.54, 0.87)	0.002
Widowed vs Married	1.67 (1.39, 1.99)	0.000
Divorced vs Married	1.49 (1.25, 1.77)	0.000
Separated vs Married	1.52 (1.18, 1.97)	0.001
Never married vs Married	1.88 (1.55, 2.28)	0.000

Notes: Reference categories as in Table A1.

Table A5: Payments (log-linear): coefficients on $\log(\text{payments} + 1)$ and implied percent change, cancer survivors.

Term	Percent change (95% CI)	p-value
Age (years)	2.6 (1.3, 3.8)	0.000
Female vs Male	15.8 (-13.0, 54.3)	0.312
Black vs White	17.0 (-21.0, 73.3)	0.430
Native vs White	-23.1 (-81.3, 216.7)	0.714
Asian/PI vs White	4.6 (-37.2, 74.3)	0.861
Multiple vs White	14.7 (-47.5, 150.6)	0.729
Public-only vs Private	83.7 (28.2, 163.1)	0.001
Uninsured vs Private	-79.6 (-87.8, -65.7)	0.000
Near-poor vs Poor	49.6 (-19.7, 178.7)	0.202
Low-income vs Poor	-27.0 (-57.7, 26.1)	0.256
Middle vs Poor	-28.8 (-51.7, 5.1)	0.087
High vs Poor	-48.9 (-68.3, -17.7)	0.006
GED vs No HS	-2.9 (-42.1, 62.8)	0.909

HS vs No HS	-31.0 (-52.0, -0.8)	0.045
Some college vs No HS	-28.3 (-55.5, 15.6)	0.170
AA vs No HS	-28.5 (-71.0, 76.2)	0.462
BA vs No HS	18.1 (-49.8, 178.0)	0.701
MA vs No HS	-13.4 (-59.9, 86.9)	0.712
Widowed vs Married	-13.4 (-42.3, 30.1)	0.485
Divorced vs Married	-35.7 (-59.9, 2.8)	0.065
Separated vs Married	-48.2 (-83.3, 60.9)	0.252
Never married vs Married	14.8 (-40.4, 121.2)	0.677

Notes: Percent change computed as $100(e^{\beta} - 1)$.

Table A6: Payments (log-linear): coefficients on $\log(\text{payments} + 1)$ and implied percent change, non-cancer adults.

Term	Percent change (95% CI)	p-value
Age (years)	1.9 (1.4, 2.4)	0.000
Female vs Male	-18.2 (-29.5, -5.0)	0.008
Black vs White	13.2 (-4.8, 34.5)	0.161
Native vs White	61.0 (-2.9, 166.8)	0.065
Asian/PI vs White	20.3 (-20.6, 82.3)	0.383
Multiple vs White	28.7 (-6.4, 77.0)	0.120
Public-only vs Private	43.3 (21.2, 69.4)	0.000
Uninsured vs Private	-34.8 (-70.0, 41.3)	0.278
Near-poor vs Poor	-28.0 (-43.3, -8.5)	0.007
Low-income vs Poor	-27.1 (-41.2, -9.6)	0.004
Middle vs Poor	-41.0 (-51.6, -28.0)	0.000

High vs Poor	-40.6 (-53.0, -24.9)	0.000
GED vs No HS	-39.0 (-55.1, -17.3)	0.002
HS vs No HS	-25.3 (-37.7, -10.4)	0.002
Some college vs No HS	-26.3 (-43.8, -3.5)	0.027
AA vs No HS	-41.3 (-58.8, -16.4)	0.003
BA vs No HS	-40.2 (-70.3, 20.3)	0.149
MA vs No HS	-35.1 (-51.6, -12.8)	0.004
Widowed vs Married	27.2 (6.0, 52.7)	0.010
Divorced vs Married	27.0 (3.1, 56.5)	0.025
Separated vs Married	20.1 (-18.0, 76.0)	0.346
Never married vs Married	85.5 (46.1, 135.4)	0.000

Notes: Percent change computed as $100(e^{\beta} - 1)$.

Appendix B: Descriptive Tables (Means of Days Among Users)

Table B1: Mean number of home health care days among cancer survivors who used HHC, by sex (survey-weighted).

SEX	Mean days	SE	95%% CI
Male	80.9	9.2	80.9 (62.8, 98.9)
Female	82.7	8.2	82.7 (66.6, 98.8)

Table B2: Mean number of home health care days among cancer survivors who used HHC, by race/ethnicity (survey-weighted).

RACE	Mean days	SE	95%% CI
White	78.6	7.7	78.6 (63.5, 93.7)
Black	105.9	14.5	105.9 (77.5, 134.3)
Native	94.9	56.5	94.9 (-16.0, 205.7)
AsianPI	27.4	14.4	27.4 (-0.9, 55.7)
Multiple	112.5	30.5	112.5 (52.7, 172.3)

Table B3: Mean number of home health care days among cancer survivors who used HHC, by age group (survey-weighted).

AGE_GROUP	Mean days	SE	95%% CI
18–44	91.3	22.8	91.3 (46.5, 136.0)
45–64	65.2	10.5	65.2 (44.5, 85.8)
65–74	61.7	11.8	61.7 (38.6, 84.8)
75–84	86.7	13.5	86.7 (60.2, 113.2)

85+	103.1	13.8	103.1 (76.1, 130.1)
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Table B4: Mean number of home health care days among cancer survivors who used HHC, by insurance coverage (survey-weighted).

INSCOV	Mean days	SE	95%% CI
Private	52.9	7.0	52.9 (39.1, 66.7)
PublicOnly	108.4	10.4	108.4 (88.0, 128.7)
Uninsured	18.2	4.7	18.2 (8.9, 27.5)

Table B5: Mean number of home health care days among cancer survivors who used HHC, by poverty category (survey-weighted).

POVCAT	Mean days	SE	95%% CI
Poor	117.7	13.6	117.7 (91.1, 144.3)
NearPoor	132.8	23.9	132.8 (85.9, 179.6)
LowIncome	87.1	19.2	87.1 (49.5, 124.7)
Middle	60.9	11.5	60.9 (38.3, 83.5)
High	55.7	9.0	55.7 (38.0, 73.4)

Table B6: Mean number of home health care days among cancer survivors who used HHC, by educational attainment (survey-weighted).

HIDEG	Mean days	SE	95%% CI
NoHS	134.5	17.1	134.5 (100.9, 168.1)
GED	65.7	15.1	65.7 (36.0, 95.3)
HS	65.2	6.0	65.2 (53.4, 77.0)
SomeCollege	75.2	17.5	75.2 (40.9, 109.5)
AA	63.3	20.3	63.3 (23.6, 103.0)

BA	16.1	5.4	16.1 (5.6, 26.6)
MA	73.8	34.1	73.8 (7.0, 140.7)

Table B7: Mean number of home health care days among cancer survivors who used HHC, by marital status (survey-weighted).

MARRY	Mean days	SE	95%% CI
Married	73.9	14.3	73.9 (45.9, 101.9)
Widowed	88.2	7.3	88.2 (74.0, 102.5)
Divorced	65.5	8.0	65.5 (49.9, 81.1)
Separated	56.4	29.5	56.4 (-1.5, 114.3)
NeverMarried	124.3	26.9	124.3 (71.5, 177.1)

Appendix C: Forest Plots

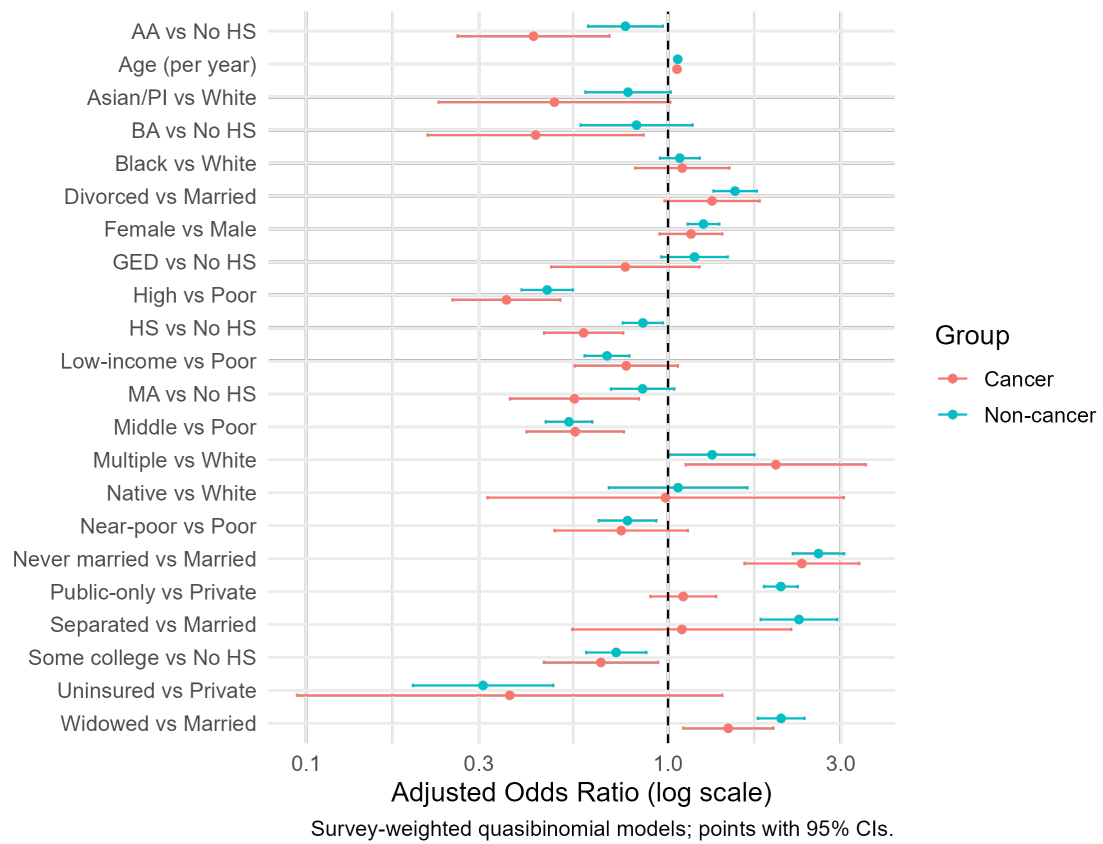


Figure C1: Predictors of any HHC use: adjusted odds ratios (log scale), cancer vs. non-cancer.

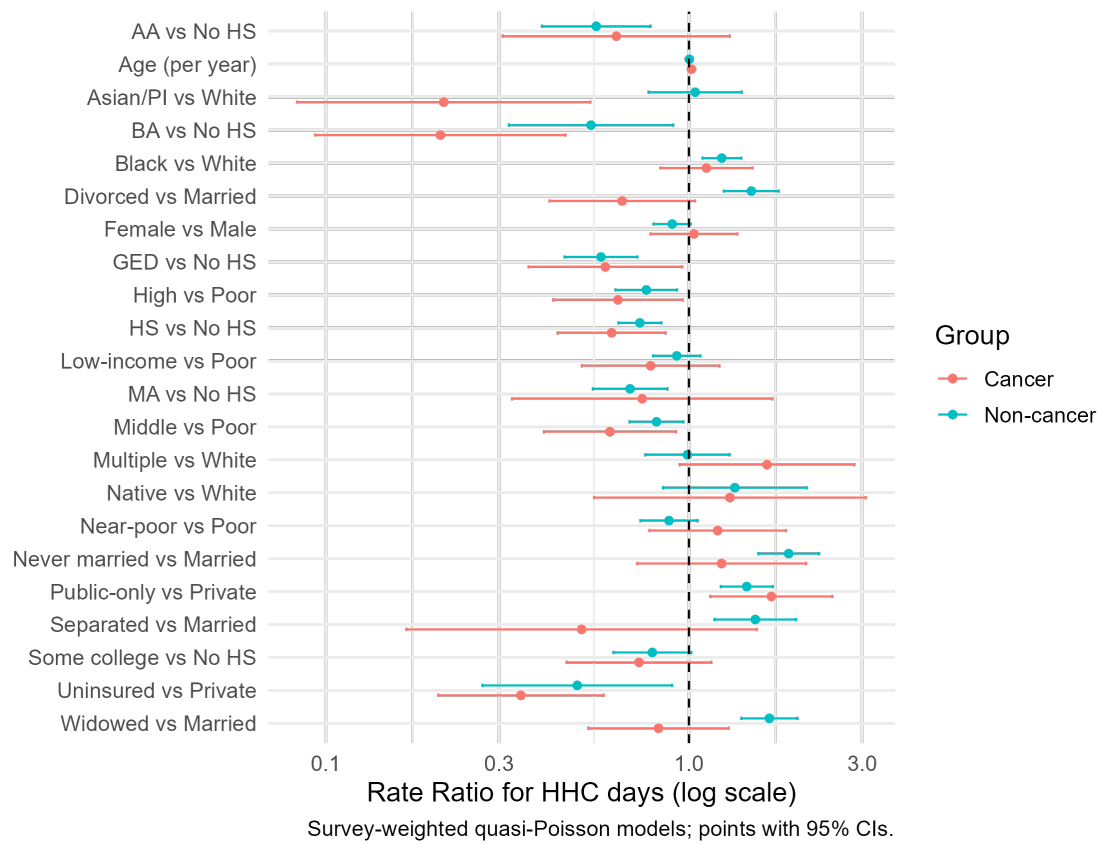


Figure C2: Predictors of intensity among users: adjusted rate ratios (log scale), cancer vs. non-cancer.

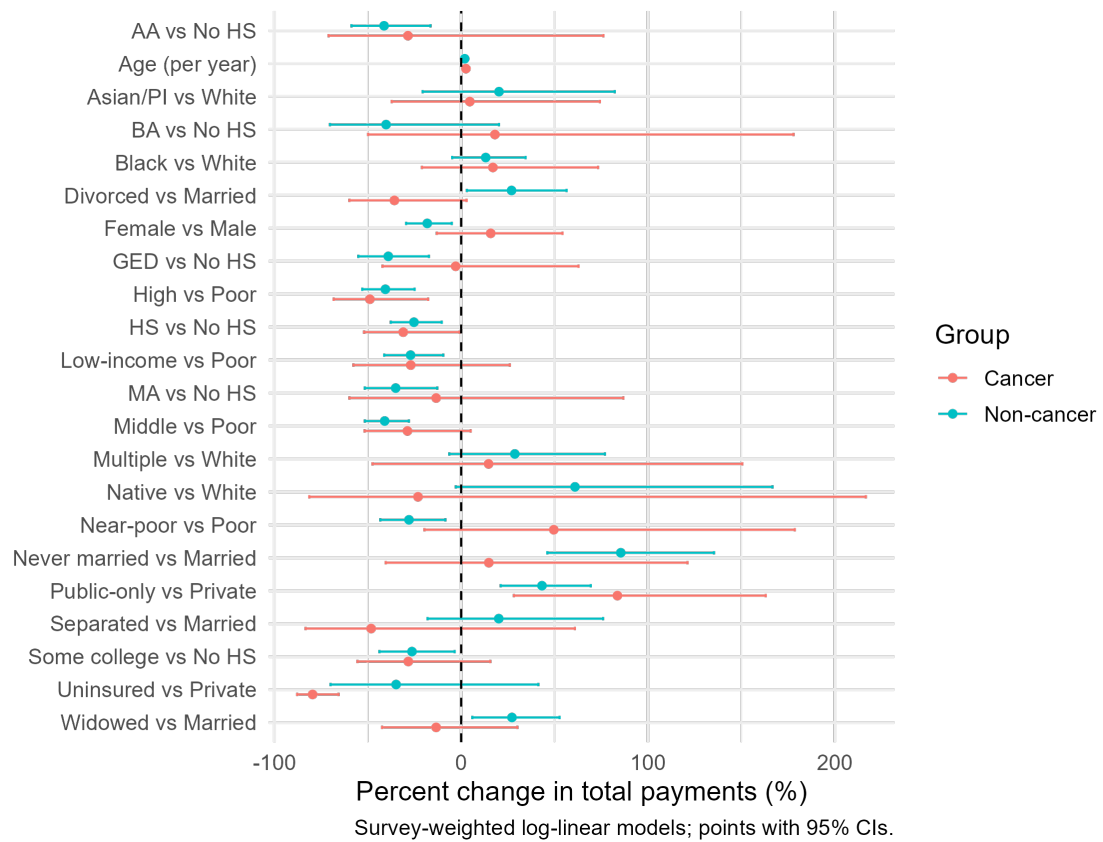


Figure C3: Predictors of payments among users: percent change in total payments, cancer vs. non-cancer.